

1. Administrative & Study Identification

Full Study Title: A Phase 1, Open-label Study of BMS-986460 in Participants With Metastatic Castration-resistant Prostate Cancer
Short Title/Acronym: A Study to Assess BMS-986460 in Participants With Metastatic Castration-resistant Prostate Cancer
Protocol Number: CA125-1008 **NCT Number:** NCT06067841 **Sponsor Name:** Bristol-Myers Squibb
Investigational Product: BMS-986460 **Phase of Development:** Phase 1 **Medical Monitor:** Bristol-Myers Squibb
Clinical Operations Lead / Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective
Primary Objective: To assess the safety, tolerability, and preliminary efficacy of BMS-986460 in men with Metastatic Castration-resistant Prostate Cancer.
Secondary Objectives: To evaluate the pharmacokinetic profile of BMS-986460 (including Cmax, Tmax, and AUC) and preliminary anti-tumor activity (confirmed PSA decline).

2.2 Background & Rationale
To address the unmet medical need in advanced prostate cancer, this study evaluates BMS-986460, a novel experimental therapy, in men whose metastatic castration-resistant prostate cancer (mCRPC) continues to grow and progress despite receiving standard hormone treatments (androgen deprivation therapy and secondary hormonal therapies).

3. Study Design & Methodology

3.1 Design Framework
Study Type: Interventional **Control Type:** Single-arm
Blinding: Open-label (None) **Randomization:** N/A (Sequential Assignment)

3.2 Planned Enrollment & Duration
Target N: 140 evaluable patients
Number of Sites: Multiple sites across the United States (including locations in CA, FL, MA, NY, NC, and WA)
Estimated Study Start: 18-Oct-2023 **Estimated Primary Completion:** 24-Aug-2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria
1. Participant must have histologically or cytologically confirmed adenocarcinoma of the prostate.
2. Participant must have an Eastern Cooperative Oncology Group (ECOG) performance status 0 to 1.
3. Participant must have

progressed on androgen deprivation therapy (ADT) and at least one prior secondary hormonal therapy approved for CRPC, with a prostate specific antigen (PSA) of ≥ 2 ng/mL at Screening.

4.2 Exclusion Criteria 1. Participant must not have a history of brain metastases. 2. Participant must not have impaired cardiac function or clinically significant cardiac disease. 3. Participant must not have any significant medical condition, including active or uncontrolled infection, psychiatric illness, or the presence of laboratory abnormalities placing the participant at unacceptable risk.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Specified dose of BMS-986460 on specified days **Route of Administration:** Systemic drug administration (per protocol) **Duration of Treatment:** Up to 112 weeks of exposure/follow-up

5.2 Concomitant & Prohibited Therapy Permitted: Background androgen deprivation therapy (ADT). **Prohibited:** Unapproved concurrent investigational agents or additional systemic anti-cancer therapies outside of the protocol specification.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, PSA Labs, Cardiac & ECOG assessment
Baseline	Day 0	First Dose, Safety Baseline
Interim	Up to Week 112	Safety Labs, Efficacy Assessment, PK sampling (intensive during first 13 weeks)
End of Study	Week 112	Final Vital Signs, Exit Interview, IP Accountability

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Number of participants with adverse events (AEs), serious adverse events (SAEs) [up to 112 weeks], and AEs meeting protocol-defined dose limiting toxicity (DLT) criteria [up to 28 days]. **Measurement Method:** Clinical and laboratory safety assessments mapped to standardized toxicity criteria.

7.2 Secondary & Exploratory Endpoints 1. Pharmacokinetics: Maximum concentration (C_{max}), Time of maximum concentration (T_{max}), and Area under the plasma concentration-time curve (AUC) up to 13 weeks. 2. Efficacy: Number of participants with a confirmed prostate specific antigen (PSA) decline of $\geq 30\%$ from baseline up to 108 weeks.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous collection of clinical and laboratory data to identify AEs leading to study intervention discontinuation. **Serious Adverse Events (SAEs):** Routine expedited reporting timeline to the sponsor. **Data Monitoring Committee (DMC):** Internal Safety Review Committee evaluating dose-limiting toxicities (DLTs) for dose escalation decisions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients who receive at least one dose of study drug) and PK-evaluable populations. **Sample Size Justification:** N=140 planned to adequately characterize safety, tolerability, DLTs, and pharmacokinetics in a Phase 1 setting. **Handling of Missing Data:** PK data handled per standard non-compartmental analysis rules; adverse events and toxicities summarized as observed.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard phase 1 onsite and remote source data verification by clinical research associates. **Audit Trail:** Assured via validated Electronic Data Capture (EDC) systems and data clarification forms.

11. Study Identifiers & Governance

Primary ID: CA125-1008 **Registry Number:** NCT06067841 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** A Study to Assess BMS-986460 in Participants With Metastatic Castration-resistant Prostate Cancer **Official Regulatory Title:** A Phase 1, Open-label Study of BMS-986460 in Participants With Metastatic Castration-resistant Prostate Cancer

12. Clinical Framework (Prostate Cancer Specific)

Primary Condition: Metastatic Castration-resistant Prostate Cancer (mCRPC) **Diagnosis Threshold:** PSA $\geq$ 2 ng/mL following progression on ADT and prior secondary hormonal therapy **Medical Methodology:** Experimental Targeted Therapy Evaluation **Optimization Target:** Identifying safe dosage levels, Maximum Tolerated Dose (MTD), and observing early PSA response

13. Study Procedures & Methodology

Management Pathway: Standard oncology Phase 1 dose-escalation/expansion clinical pathway. **Education Protocol (HCP):** Site initiation visits and comprehensive training on DLT criteria and PK sampling. **Education Protocol (Patient):** Informed consent, onsite training regarding drug safety, and treatment compliance. **Quality Audit Mechanism:** Safety review meetings and clinical site monitoring.

14. Observation & Follow-Up Schedule

Total Duration: Up to 112 weeks **Onsite Visit Cadence:** Intensive monitoring during early cycles (up to 13 weeks for PK and 28 days for DLT evaluation), followed by standard oncology intervals. **Remote Monitoring Frequency:** As per site-specific clinical monitoring plans. **Checklist Review Interval:** Regular review prior to each dosing cycle for AE and DLT assessments.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				